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/ **Practice Exam - 1**

Correct Answer

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Practice Exam - 1

Question 57 of 117



A 55 year old man who is status post orthotopic heart transplant returns to clinic for initial follow-up following transplantation. His immunosuppressant regimen consists of tacrolimus 5 mg twice daily (tacrolimus trough: 8 ng/ml), mycophenolate mofetil 1000 mg twice daily , and prednisone 20 mg daily . The patient demonstrates both resting and intention tremors. The tremors are impacting the patient's activities of daily living, requiring supervision for eating and some assistance for grooming, bathing, and dressing.

Which of the following interventions would be the most appropriate to treat his tremors?

- A. Switch tacrolimus to cyclosporine modified.
-  B. Reduce the dose of MMF to 750 mg twice daily
- C. Add oxycodone 10 mg every 4-6 hours as needed.
-  D. Add primidone 50 mg daily and titrate to 250 mg/day

Commentary

References

Testing Point: The Board-certified AHFTC specialist should be able to identify and manage common adverse medication side effects in heart transplant recipients.

Answer: D. Add primidone 50 mg daily and titrate to 250 mg/day.

Tacrolimus and cyclosporine, both calcineurin inhibitors, can cause significant neurological side effects. Tremor is the most common tacrolimus-associated side effect and can also be seen with cyclosporine. In the case of this patient, primidone has been shown to be safe and effective at reducing tremor by 60% 1 to 7 hours after ingestion, with stable serum primidone levels but no detectable phenobarbital levels at doses of 250 mg/day. 15-25% of the primidone is metabolized to phenobarbital. However, no drug-drug interactions would be expected with the patients CNI regimen. The addition of primidone to this patient's regimen could reduce his tremors and improve his quality of life, making Answer D correct.

Primidone levels should be monitored on therapy and maintained between 5 and 10 mg/L. While switching to cyclosporine modified could be considered, the patient is a fresh transplant and is currently at his goal tacrolimus trough, making Answer A not the best option.

Reducing the dose of MMF will not address his tremors and again, he is newly transplanted making Answer B as incorrect.

Adding an opiate is not recommended for the management of tremors and can lead to hazardous side effects, thus Answer C is not the best choice.

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